

# Overview of Developmental Milestones and Needs

with Mariah Rooney and Kai Cheng Thom



# ACKNOWLEDGEMENTS



The background of the slide is composed of a central white horizontal band. Above and below this band are solid brown sections. White, hand-drawn style wavy lines are scattered across the brown sections, with some lines extending from the top brown section into the white band and others from the bottom brown section into it.

# LANDING + ORIENTING



HELLO. INTRODUCTIONS.



# AGENDA

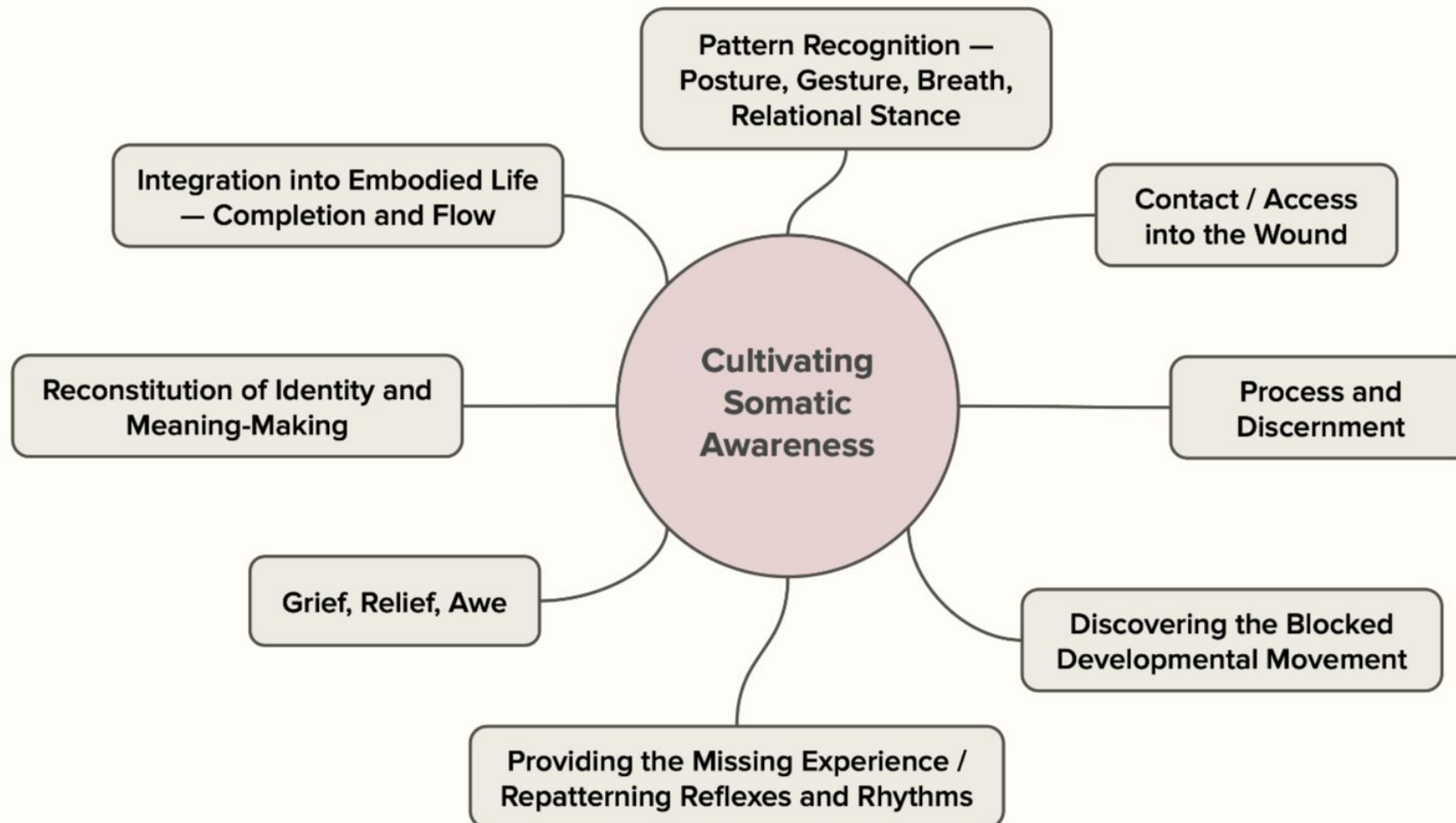
Relational Development: A Somatic  
Attachment Lens  
Ecosystemic Lens  
Assessment  
Demo



# RELATIONAL DEVELOPMENT:

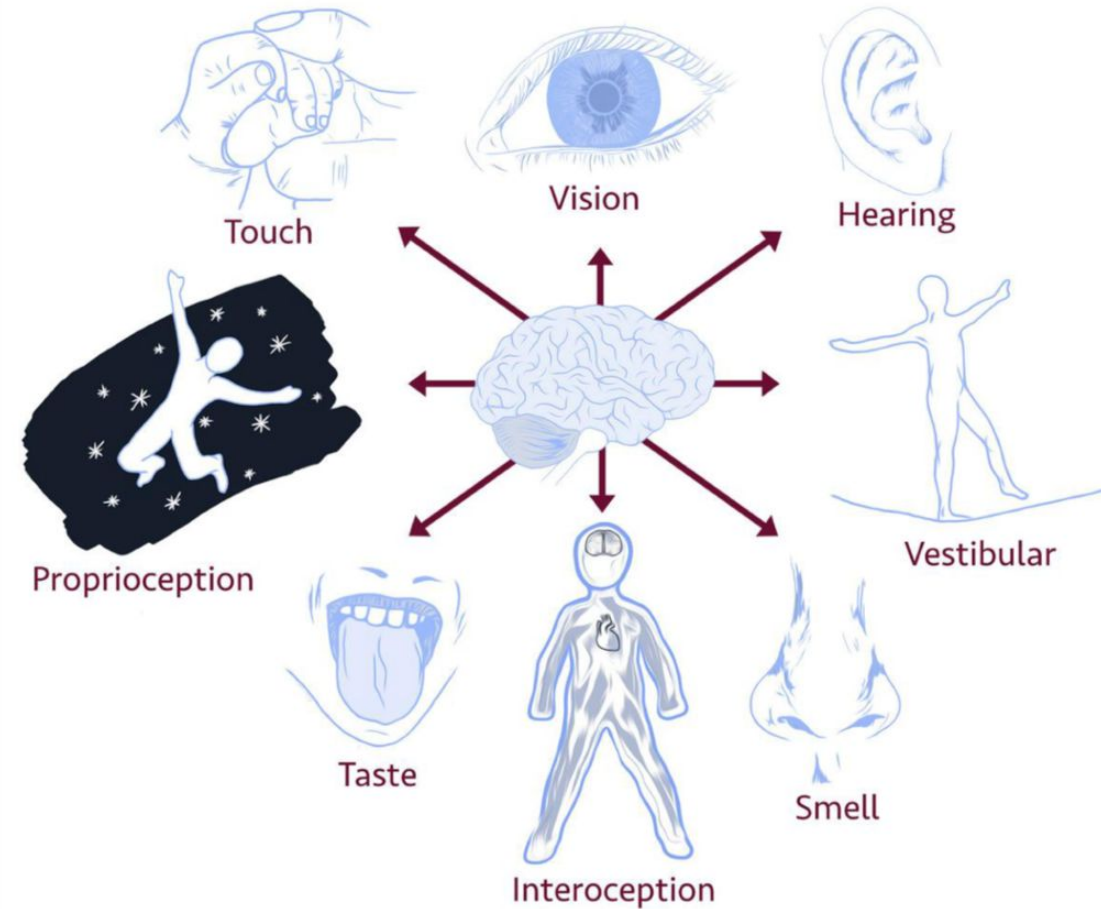
*A SOMATIC ATTACHMENT LENS*

# The 8-Phase Map of Developmental Repair



# SENSORY PROCESSING & ATTACHMENT

- Did you know that you have not only got 5 sensory systems, but 8?
  - There are 3 other senses which are more internal and tell us about our body

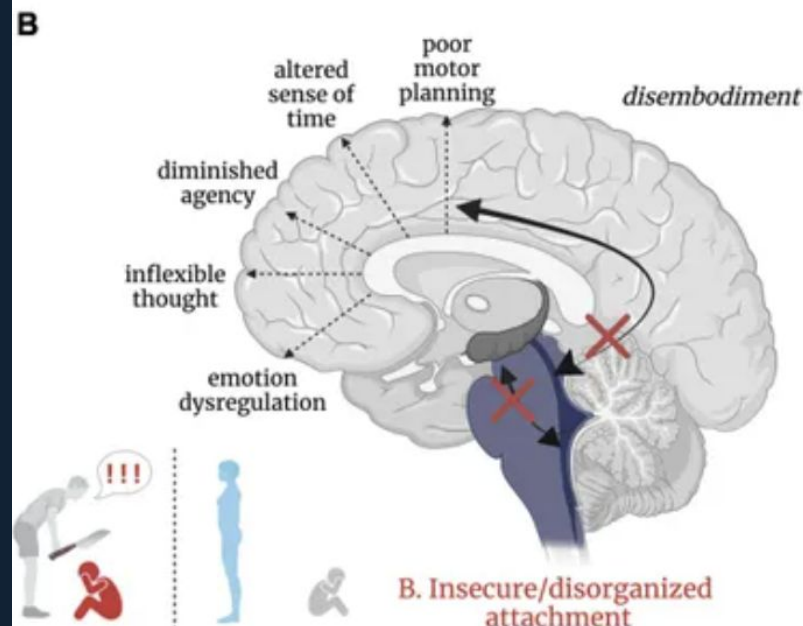
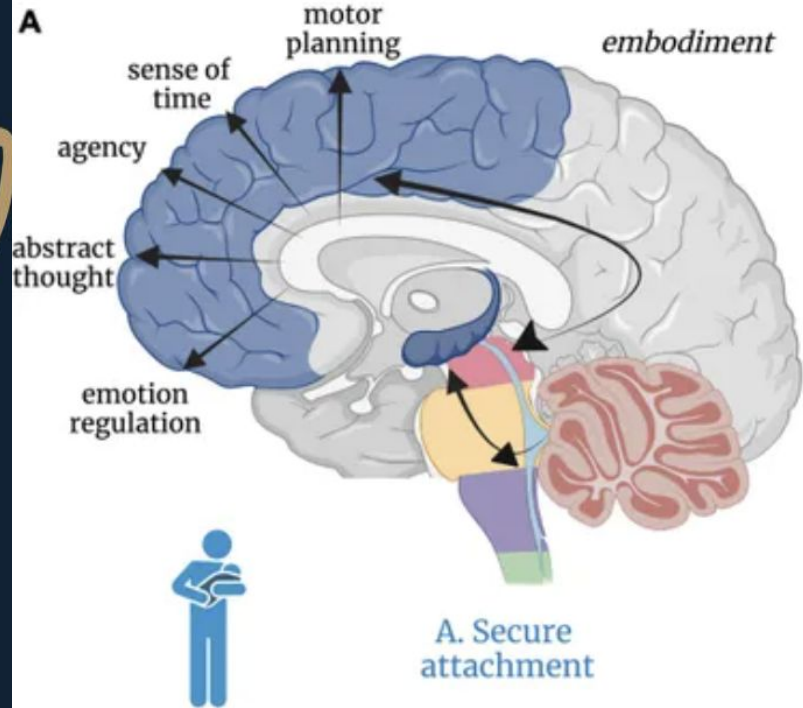




# IMPACTS OF SOMATIC SENSORY PROCESSING

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**Figure 5. (A,B)** Good versus poor somatic sensory integration during development with a secure versus insecure attachment figure, respectively. **(A)** Healthy brainstem level multisensory integration as a result of attuned, nurturing, somatic sensory-rich caregiving provides the foundation for higher-order limbic and cortical capacities. **(B)** An overwhelmed or malnourished brainstem due to abusive or neglectful caregiving. The child does not receive comforting touch or attuned rhythmical movement with a safe affective caregiver. Somatic sensory experiences are paired with unsafe or insecure attachment relationships, which has cascading aversive effects on higher-order limbic and cortical capacities.

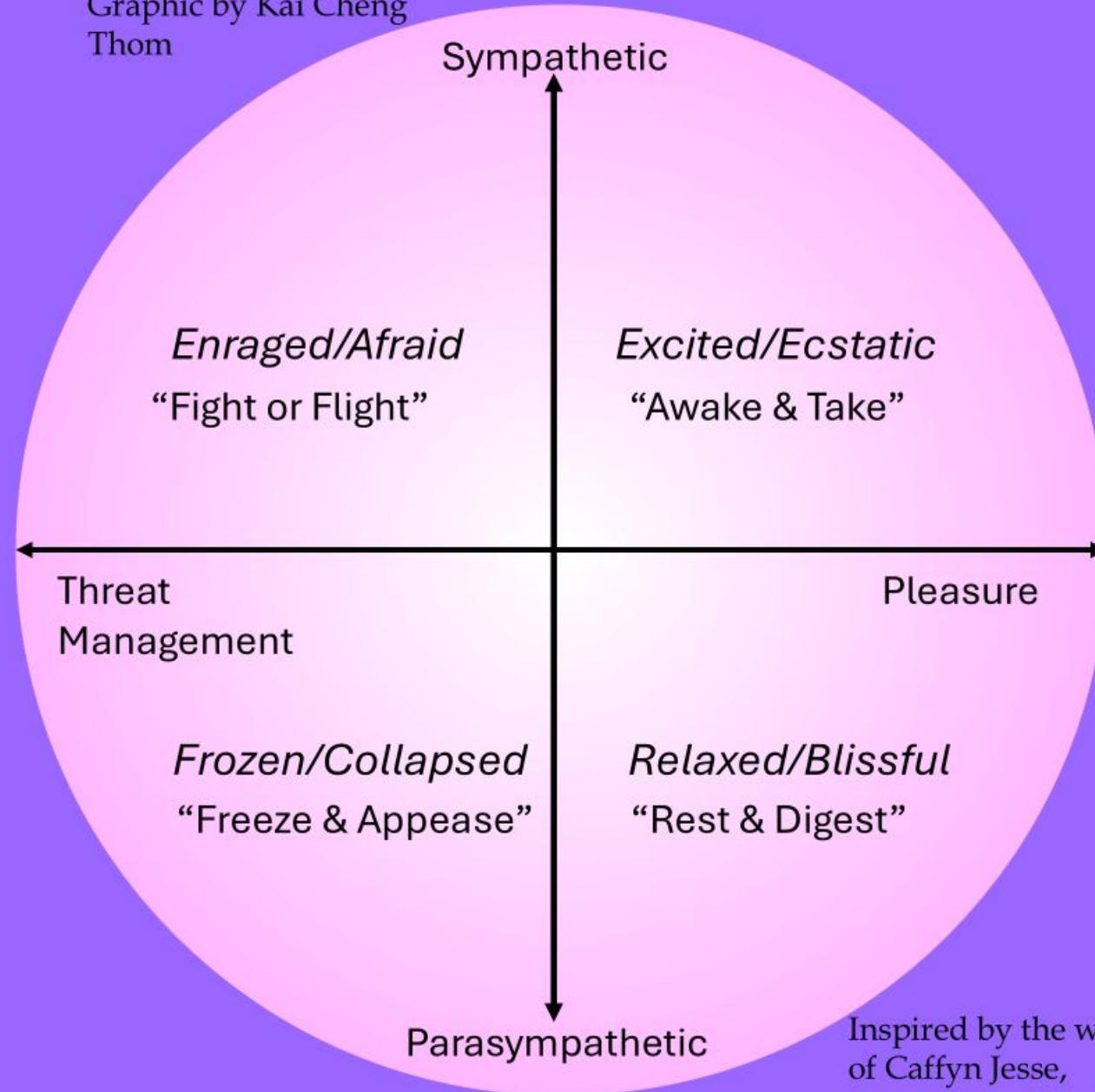
# BASIC NEEDS: A DIFFERENT LENS

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- Both literal and symbolic
  - If basic needs met: Pleasure, satisfaction, meaning, connectedness
  - If not met: Pain, frustration, meaninglessness, alienation/isolation
- **Place**
  - **Nurturance**
  - **Protection**
  - **(Flexible Loving) Limits**
  - **Support**

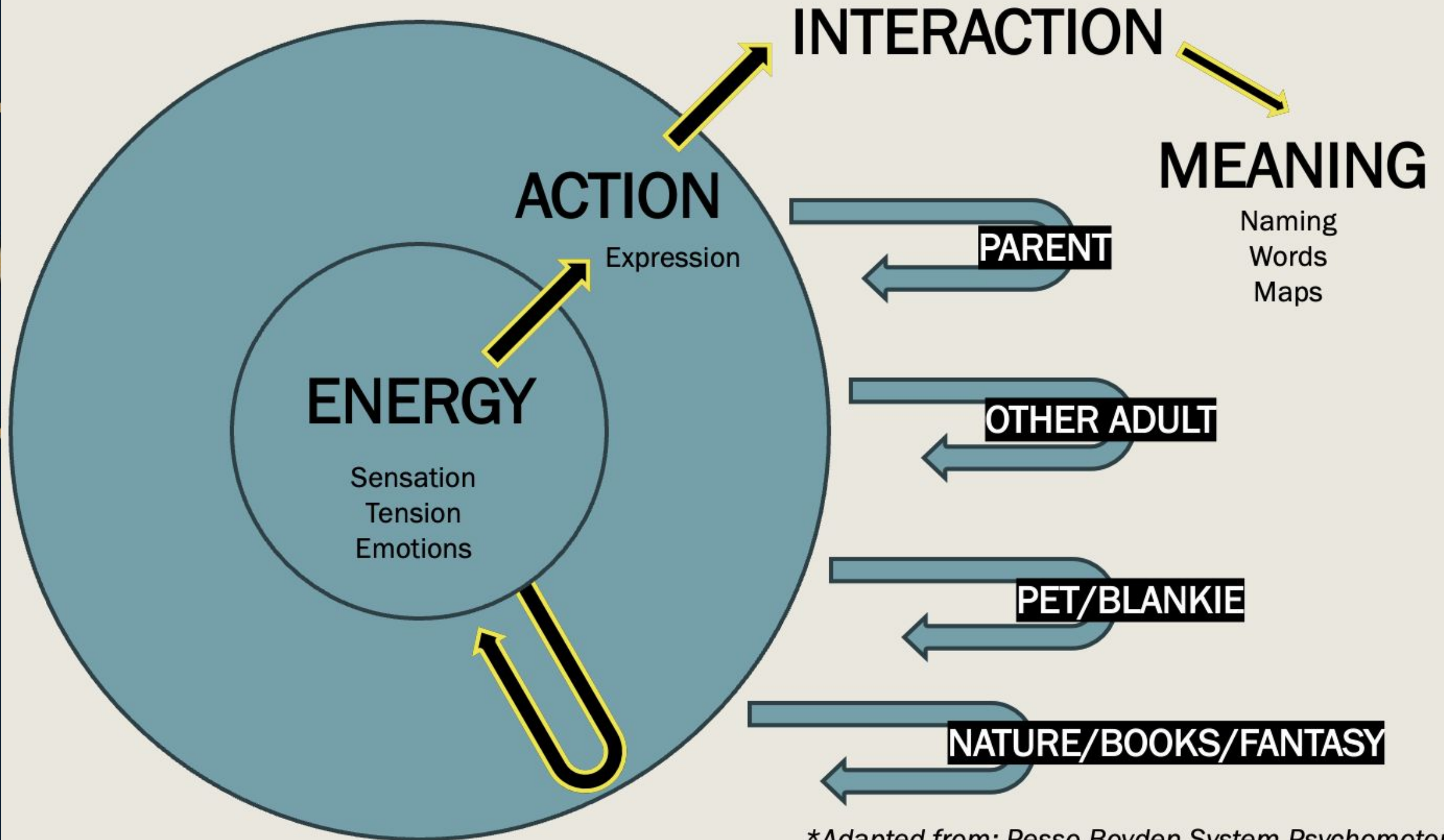
# Nervous System States, Survival & Pleasure

Graphic by Kai Cheng  
Thom



Inspired by the work  
of Caffyn Jesse,  
Somatic Sex Educator





*\*Adapted from: Pesso Boyden System Psychomotor*

# MESSAGES FROM CAREGIVERS, RELATIONSHIPS & SOCIETY IMPACT THE BODY

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- "You are not wanted"
- "You belong and are safe"
- "There is no time for you"
- "You must do what I say"
- "You are special to me"
- "You have choices"
- "Your feelings don't count"
- "Your feelings are important to me"

- "Be strong, not needy"
- "You can depend on me"
- "It's your fault I'm unhappy"
- "You are not responsible for my upset"
- "You have to perform"
- "You are loved for who you are"
- "You are not loveable"
- "You are welcome"

# Spheres of Embodied Healing

By Kai Cheng Thom©



The background of the slide is a solid brown color. It is decorated with several white, hand-drawn style wavy lines. One line starts in the top right corner and curves downwards. Another line starts in the bottom left corner and curves upwards. A third line starts in the bottom right corner and curves upwards. These lines create a sense of movement and depth.

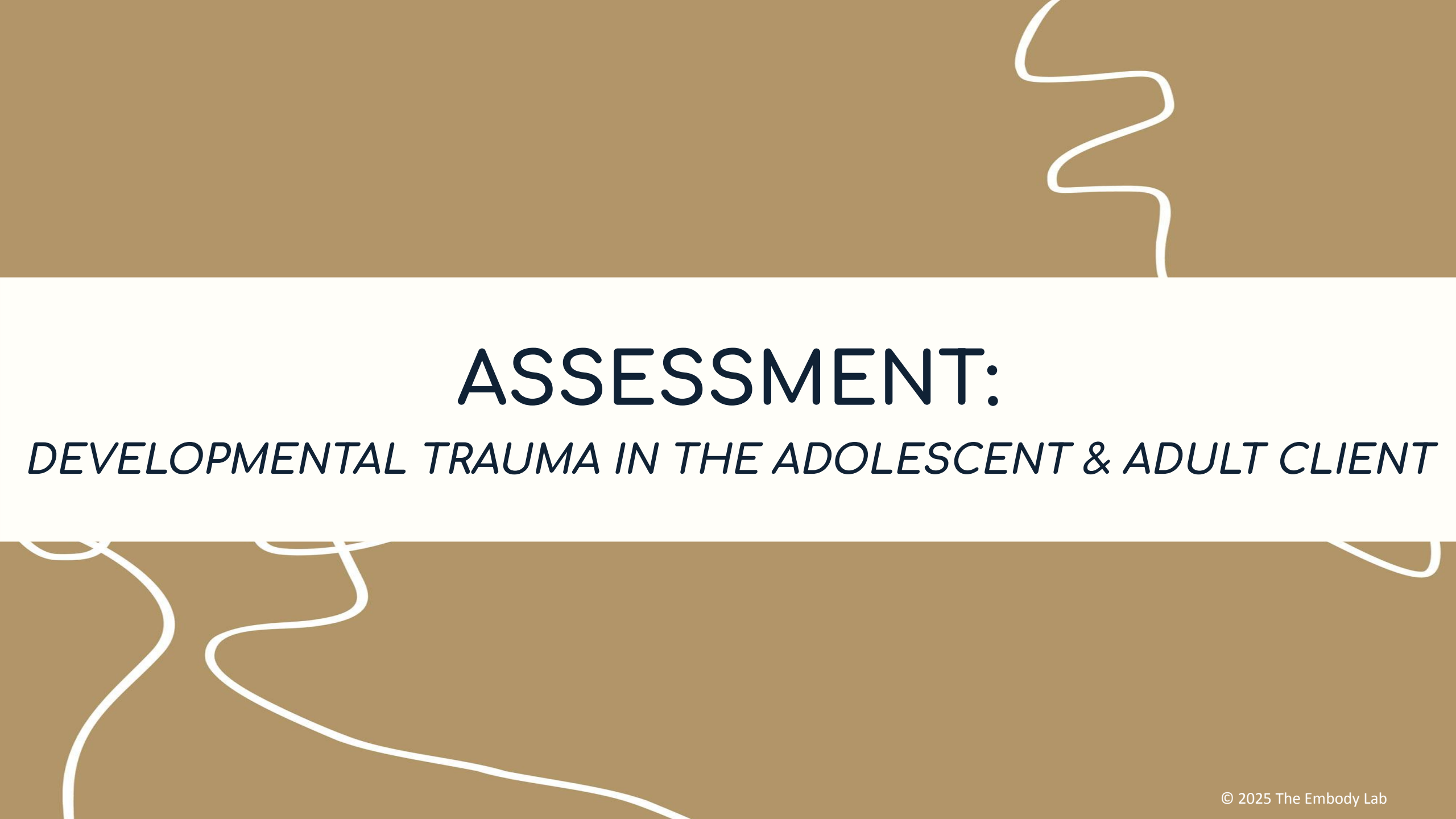
# ECOSYSTEM LENS FOR ATTACHMENT AND TRAUMA



# ECOSYSTEMIC LENS: ATTACHMENT & DEVELOPMENT

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- Attachment as a model has historically been rooted in a psychodynamic, dyadic model of relationality (mother & child; adult intimate partner & adult intimate partner)
- This fits neatly into a heterosexual, nuclear family-centered understanding of how relationships form and impact the developing mind, brain, and soma
- What is left out? Cultural models of care and child-rearing that centre extended kinship networks and broader webs of relationality, importance of intergenerational cultural transmission and the impact of broader cultural systems on internal working models of self/other - *“Each night, without fail, she prayed for blue eyes. Fervently, for a year she had prayed. Although somewhat discouraged, she was not without hope. To have something as wonderful as that would take a long, long time.”* – Toni Morrison, *The Bluest Eye*
- There can be grave impacts when psychological professionals do not account for the ways in which social systems themselves are a interrelational force that exerts influence on the intrapsychic space, and in that shape our own biases



# ASSESSMENT:

*DEVELOPMENTAL TRAUMA IN THE ADOLESCENT & ADULT CLIENT*

# DEVELOPMENTAL TRAUMA DISORDER

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- Diagnosis was proposed for inclusion in the DSM-5
- Consensus proposed criteria for DTD:
  - Exposure
  - Affective and physiological dysregulation
  - Attentional and behavioral dysregulation
  - Self and relational dysregulation
  - Posttraumatic spectrum symptoms
  - Duration of disturbance at least 6 months
  - Functional impairment (scholastic, familial, peer group, legal, health, vocational)

\*D'Andrea et al., 2012

# FRAGMENTATION OF EMOTIONAL EXPERIENCE

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- **Interrupted affect development:** Attachment ruptures disrupt the child's ability to sense, name, and regulate emotions, leaving scattered or unsatisfying affective experiences and capacities
- **Somatic-emotional splitting:** Sensations, emotions, and meaning-making become uncoupled...the body feels one thing, the mind reports another
- **Overcoupling & undercoupling patterns:** Emotions may fuse with unrelated sensations (overcoupling) or disconnect entirely from bodily experience (undercoupling), leading to confusion or emotional numbness
- **Disrupted access to emotions:** Feelings arise with intensity, delayed, or not at all...experienced as “too much,” “not enough,” or “not mine”
- **Developmental arrest in affect:** Certain emotions remain developmentally thwarted or frozen at the age they were overwhelming or unsupported (or shamed, ignored, policed, punished, etc.)
- **Context-bound states:** Emotions become siloed...accessible only in particular environments, relationships, or situations/contexts
- **Protective/adaptive fragmentation:** Disowned or compartmentalized emotional states emerge as adaptive strategies or parts of self

# PRACTICING DISCERNMENT

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- Differential conceptualization and understanding
- Caution against over-assigning/mis-assigning value or explanation to trauma
- “TikTokification” of mental health
- Trauma-informed care is *collaborative*
  - Practice and embody humility, signal invitation and invite empowered choice and voice
  - Offer the client an “interpretation,” a “reflection” - “tell me what you think about this? How is this landing with you?”
  - Have a strong referral network
  - Develop capacities to offer psychoeducation around other possibilities in terms of conceptualization and care

# Holding Complexity

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- Medical models of mental health have traditionally been quick to assign “personality disorder” and “characterological” diagnoses to presentations that appear to lack coherence - ex, rapid and drastic mood swings, affectively warm/charming but internally shut down or rageful, emotional numbness punctuated by bursts of relational intensity, “lacking a sense of self,” etc
- Many contemporary models of somatic and developmental trauma healing invite us to hold the complexity of an individual’s inner landscape with an orientation towards the resilience and safety-seeking wisdom of supposedly incoherent presentations -we can conceptualize them as survival strategies or “safety shapes” that have been “frozen in time” and are operating simultaneously - this is connected to “parts work” theory and approaches
- One aspect of the soma is moving in one direction and another aspect is moving in a different direction
- Important for practitioners to notice and hold our own complexity - where do we bring contradiction and polarity into the work?





# QUESTIONS



# DEMONSTRATION



# BREAKOUT SESSIONS

# Discussion Prompts

1. Where do you notice early relational imprints showing up in your clients' somatic patterns...posture, affect, timing, tone, or pacing?
2. Where do you feel confident discerning developmental trauma from other psychological and/or systemic factors? Where do you feel less sure?
3. What helps you hold curiosity instead of certainty when conceptualizing complex presentations?
4. How do cultural, systemic, and intergenerational factors show up in the body or emotional life of your clients?